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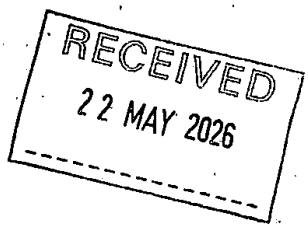
Southwark Low Intensity Team (LIT)
1st Floor, Marina House
63-65 Denmark Hill
London
SE5 8RS
Telephone: 0203 228 9454
Email SouthwarkLIT@slam.nhs.uk

Date: 18 May 2026

Private & Confidential

To:
The Acorn & Gaumont House Surgery
153 Peckham High Street
Peckham
London
Greater London
SE15 5SL

Patient Details:
Mayaki, Alfred (Mr)
Born 18/12/1987 (38y)
NHS No. 625 082 5304
Tel. 07940916731.



Dear Sir/Madam,

Re: Transfer of Care to Primary Care

DIAGNOSIS:
F25.0 - Schizoaffective disorder, manic type.

Following our previous correspondence and the Shared Care Agreement issued by the Consultant after a telephone discussion, we are writing to formally transfer the care of above patient to primary care.

Clinical Background
This is a 38-year-old man with a diagnosis of schizoaffective disorder, manic type. He had psychiatric admissions under Mental Health Act.
He has remained stable in his mental state for a substantial period.
He is currently compliant with Aripiprazole depot, with good adherence to treatment.

Current Mental State

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On recent reviews, he has consistently presented as:

- Calm and cooperative
- Bright in mood
- Maintaining good eye contact
- Demonstrating good self-care and personal hygiene

He has **not expressed any suicidal ideation**, and there are no concerns regarding immediate risk to self or others at present.

Engagement
The patient continues to engage well with the Care Coordinator (CC) and Low Intensity Team (LIT) and has demonstrated a positive level of insight and cooperation with services.

Ongoing Concerns
Lifestyle factors have been discussed at length, including diet, physical activity, and metabolic health. However, he will require ongoing proactive monitoring and support to ensure effective implementation of these interventions.

Medication:
Medication Aripiprazole, Dosage 960mg, 2 Monthly.
Next depot due 09/06/2026

RISK

TO SELF
No thoughts of harm to self. No signs of self-neglect.
Historical: when unwell self neglects, social isolation.

TO OTHERS
No thoughts of harm to others.

Historical: when unwell can be hostile, sexually inappropriate. Has broken into a house and physically assaulted a woman when acutely psychotic.

FROM OTHERS
Nil risk from others identified.

RELAPSE PREVENTION / CRISIS PLAN
Engagement with Services
Depot compliance
Routine, good structure with in-built flexibility
Supportive family
Crisis line

Recommendations for Primary Care
We would recommend the following:
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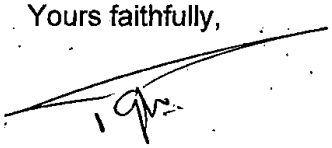
- Continuation of Aripiprazole depot as per Shared Care Agreement
- Regular physical health monitoring,
- Ongoing support for lifestyle modification, including weight management interventions
- Monitoring for any early signs of relapse in mental state

Follow-Up
The patient is now considered suitable for management within primary care. Should there be any deterioration in mental health, please do not hesitate to refer back to secondary care services.

Conclusion
In summary, this patient is clinically stable, compliant with treatment, and engaging well with services. With appropriate monitoring and support, he is suitable for ongoing management under primary care.

Please do not hesitate to contact us if you require any further information.

Yours faithfully,



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CC Patient

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